



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : ام الهنا عبد المعطي بيومي عبدالمحسن

المحافظة : القاهرة الحي / المدينة : حلوان

رقم البطاقة : 27404010105141 رقم التليفون : 01126209098

العمر (تاريخ الميلاد) : 01-04-1974 المهنة : ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإحصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
الاسم : د/ندى حسن الوظيفة : التوقيع :
الاسم : د/نرمين رمضان الوظيفة : التوقيع :

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بان البيانات المدونة في هذا النموذج صحيحة.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... د/..... د/..... د/..... د/.....

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى : قسم :
جهة العلاج : نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: ام الهنا عبد المعطي بيومي عبدالمحسن National ID: 27404010105141	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة توثيق الشخصية



ام الهنا
عبدالمعطي بيومي عبدالمحسين
من شعراوي - حدائق حلوان
المعصرة - القاهرة

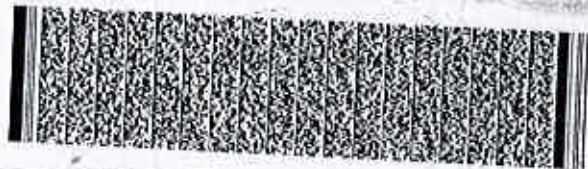
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حاصلة على دبلوم تجارة

أنثى
مسجلة
متروجة
حجاج رمضان بن محمد محفوظ
البطاقة مارية حتى ٢٠٢٤/٠٤/٠١



Patient Name :
Patient No. : 27404010105141
Path No. : 1018/2022
Sex : Female
Age : 48 Y
Referring Doc. :

Request No. : 13007700
Request Date : 16/04/2022
Verification Date : 26/04/2022
Financial Category : Out Patient
Department :
IHC No. :
Registered by : marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

Clinical data:
Left breast lesion

Nature of Specimen :
Tru cut biopsy

Gross:
Multiple grayish white tissue cores, longest measured up to 1 cm long. Totally submitted.
Sections prepared from paraffin block for immune- staining using Bench Mark ULTRA system for ER, PR, HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9).

Microscopic:
histopathological examination of sections prepared from the specimen received reveals cores of an infiltrating malignant epithelial growth. The tumour is made up of groups and small sheets of pleomorphic hyperchromatic malignant ductal epithelial cells. Stromal desmoplasia is moderate. Cellular and nuclear atypia are moderate [grade II].

Biomarkers:
ER: Strong nuclear reaction in 70% of tumor cells (score 8/8)
PR: Strong nuclear reaction in 70% of tumor cells (score 8/8)
Her-2: Weak incomplete membrane reaction of tumor cells (score 1+)
Ki 67: 10 % nuclear reaction

Diagnosis:
breast, Tru cut biopsy,
invasive duct carcinoma, GIL
positive (8/8)

Verified by :
Dr.Marwa mahmoud

Date : 08/05/2022 09:30:05
Page 1 of 2

Printed By : marwa mohamed yousse

Patient Name : الأستاذة / أم الهنا عبدالمعطي بيومي عبدالمحسن
Patient No. : 27404010105141
Path No. : 89/2023
Sex : Female
Age : 48 Y
Referring Doc :
Request No. : 13003368
Request Date : 11/01/2023
Verification Date : 24/01/2023
Financial Category : نفقة الدولة - اورام دار السلام
Department : Out Patient
Registered by : marwa mohamed youss

Pathology Report

Left breast cancer, post chemotherapy; Left simple mastectomy and SLNs biopsy specimens:

- Invasive tubular carcinoma, grade I
- Free nipple and areola
- Free surgical margins
- Positive metastatic deposits in 1/4 SLNs with capsular invasion
- Pathologic partial response
- Stage= ypT2 ypN1

Prof.Dr. Eman Gouda
Dr. Heba Mostafa

Verified by :
walaa ghoniem

Printing Date : 24/01/2023 12:33:55

Page 2 of 2

Printing By : marwa mohamed youss

Patient Name	: الأستاذة / أم الهنا عبدالمعطي بيومي عبدالمحسن	Request No.	: 13003368
Patient No.	: 27404010105141	Request Date	: 11/01/2023
Path No.	: 89/2023	Verification Date	: 24/01/2023
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 48 Y	Department	: Out Patient
Referring Doc	:	Registered by	: marwa mohamed youss

Pathology Report

Clinical Data: Left breast cancer, post chemotherapy.

Nature of the specimen: Left simple mastectomy and left SLNs biopsy specimens.

Gross Examination:

- Procedure: Left simple mastectomy specimen; measured 24x16x7 cm, covering skin measured 17x13 cm, nipple and areola were unremarkable grossly.
- Localization: LIQ
- Tumor extent
 - o Number: Ill defined rubbery grayish white TBS with central firm area
 - o Size: 5x3x2 cm.
 - o Skin involvement: 1.5 cm from skin
 - Distance from margins: 2 cm away from deep margin
- Lymph nodes number and size: Three fibrofatty tissue pieces ranging in size from 3x2 cm up to 4.5x2.5 cm, entangling 4 lymph nodes up to 1x1 cm.
- Additional pathologic findings: N/A
- Immunohistochemistry technique: N/A
- Frozen section: Four lymph nodes with therapy effect for paraffin examination.

Microscopic examination:

- Histological type: Invasive tubular carcinoma, cellularity 60%
- Grade (differentiation + nuclear pleomorphism + mitosis): I (2+2+1)
- Lymph node involvement: Positive metastatic deposits in 1/4 SLNs with capsular invasion
- Resection Margins: Free
- Histologic response to therapy: Pathologic partial response
- Additional pathologic findings: Free nipple and areola

Diagnosis:

Verified by
walaa ghoniem

Printing Date : 24/01/2023 12:33:55

Page 1 of 2

Printing By : marwa mohamed youss

Patient name: Om El Hana Abd El Aty
Date: 10.4.2022

**Full Field Digital bilateral breast mammography &
Complementary Ultrasound**

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO DBT (Digital Breast tomosynthesis)

Report:

- Heterogeneously dense breast parenchyma with nodulation and mottling impressive of adenosis (ACR C)
- No suspicious micro calcifications or architectural distortion detected.
- Bilateral scattered variable sized cysts some of them show echogenic content.
- Left breast 2 o'clock ill defined area of altered parenchyma echogenicity with tracts and pockets extending towards the skin showing echogenic internal content extending to the nipple.
- Normal skin thickness, and nipple-areola complex
- Nonspecific axillary LN.

Opinion:

- Bilateral fibrocystic mammary changes.
- Left breast 2 o'clock pathological process likely inflammatory rather than being neoplastic for histopathological assessment (BIRADS 4a)

Best regards,
Dr. Aya Deabes, MSc.



Patient Name: Om Alhana Abd Al Moaty Bayomy Abd Al Mohsen	Date : 24/12/2022
Exam : MAMMOGRAPHY BOTH	Age : 48 Year(s)
Medical ID: 1398/2022	Pat. ID: 14953

**BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION
ULTRASOUND**

Indication:

Left BC, on neoadjuvent

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO views DBT (Digital Breast Tomosynthesis).
- Bilateral breast ultrasound

Report:

- The breasts are heterogeneously dense which may obscure small lesions (ACR C)
- Left 2:00 spiculated lesion measuring 2.2x2.2 cm seen 5.2 cm from the nipple, surrounded with multiple well-circumscribed lesions of high density. US showed 2:00 spiculated hypoechoic solid lesion measuring 1.34x1.67 cm, with multiple dilated ducts extending from the nipple toward the lesion, showing internal echogenic contents.
- No right suspicious mass lesions, suspicious micro calcifications or architectural distortion detected.
- Complementary ultrasound showed no right solid mass lesions.
- Bilateral multiple scattered anechoic simple cysts, with few in the left breast showing internal echogenic contents
- Normal skin thickness and nipple-areola complex.
- Bilateral non-specific axillary LNs.

Opinion:

- Left UOQ subtle progress response of UOQ lesion (BIRADS6)
- Left Retroareolar dilated ducts with ?? inspissated secretions (BIRADS3)
- Bilateral fibrocystic changes (BIRADS 2)

Yours truly

Signature Dr. Yasmin Nada

24/12/2022 11:03 AM



Name: om el hana abd el moaty. Date: 14.06.2022

Contrast enhanced Spectral Mammography
CLINICAL SUMMARY : left breast IDC grade II

TECHNIQUE: CC & MLO low energy & recombined (post contrast) images
FINDINGS :
Background Uptake: Mild

LEFT BREAST:

- Pre-contrast images: UOQ area of asymmetry.
- Post-contrast images:
 - an area of non-mass enhancement is seen at the UOQ, measures 11 x 8 x 3 cm in diameter.
- Normal skin / nipple / areolar complex.
- Pathological axillary lymph nodes (at least 3) level I.

RIGHT BREAST:

- Dense fibroglandular pattern (ACR C).
- No masses, asymmetry or distortion or suspicious microcalcifications
- Benign calcific foci are seen.
- Post contrast images show, no pathological uptake.
- Normal skin / nipple / areolar complex.

IMPRESSION :

LEFT BREAST: (BIRADS 6= Pathology proved malignant) UOQ malignant non mass enhancement.

RIGHT BREAST: (BIRADS 1= Negative) normal study

Dr Hanaa Abou El Magd

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

1. Chest:

- Diffuse pathological nodular enhancement of the left breast. Associated few prominent left axillary LNs are seen reaching 1.3 cm.
- Few bilateral tiny subpleural pulmonary nodules are seen (2-3 mm).
- Bilateral LL, posterior subpleural fine fibrotic bands.
- Otherwise, No sizable pulmonary masses, areas of consolidation or cavitary lesions.
- Few prominent superior mediastinal (pretracheal and retrosternal) lymph nodes are seen, largest measures 1 cm likely reactive.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Right thyroid lobe, hypodense nodule is seen measuring 1x 0.7 cm.

2. Abdomen and pelvis:

- The liver is mildly enlarged with homogenous CT texture. No focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Bulky spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no back-pressure changes. Right renal upper calyceal tiny hyperdense gravel.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Under filling of the urinary bladder with no stones.
- Normal appearance of the other pelvic organs. IUD is seen in place.
- No abdominal or pelvic lymphadenopathy or collections.
- No suspicious lytic or sclerotic bone lesions.

OPINION:

- Diffuse pathological nodular enhancement of the left breast with associated few prominent left axillary lymph nodes, for sono-mammographic correlation.
- Few bilateral tiny subpleural pulmonary nodules ; likely insignificant. Otherwise, no definite pulmonary metastatic deposits.
- Mild hepatomegaly. No focal lesions.
- No suspicious lytic or sclerotic bone lesions.

Dr. m. gomme, mg
Dr. Peter Fahmy, MSc



مبادرة رئيس الجمهورية
لدعم صحة المرأة المصرية

الست المطرية
هنا
طحة
مطر

MDT FORM 11 / 2 / 2023 GB

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د. هادي القاسم	د. هادي القاسم	د. هادي القاسم		د. هادي القاسم	

Patient Name: Om Alhana Abd Al Moaty Bayomy Abd Al Mohsen	Date : 24/12/2022
Exam : MAMMOGRAPHY BOTH	Age : 48 Year(s)
Medical ID: 1398/2022	Pat. ID: 14953

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Signature Dr. Yasmin Nada

24/12/2022 11:03 AM



Patient Name : الأستاذة / أم الهنا عبدالمعطي بيومي عبدالمحسن	Request No. : 13003368
Patient No. : 27404010105141	Request Date : 11/01/2023
Path No. : 89/2023	Verification Date : 24/01/2023
Sex : Female	Financial Category : أورام دار السلام - نفقة الدولة
Age : 48 Y	Department : Out Patient
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- Resection Margins: Free
- Histologic response to therapy: Pathologic partial response
- Additional pathologic findings: Free nipple and areola

Diagnosis:

Verified by :
walaa ghoniem



Patient Name	: الأستاذة / ام الهنا عبدالمعطي بيومي عبدالمحسن	Request No.	: 13003368
Patient No.	: 27404010105141	Request Date	: 11/01/2023
Path No.	: 89/2023	Verification Date	: 24/01/2023
Sex	: Female	Financial Category	: أورام دار السلام - نفقة الدولة
Age	: 48 Y	Department	: Out Patient
Referring Doc :		Registered by	: .marwa mohamed youss

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Prof.Dr. Eman Gouda
Dr. Heba Mostafa

Verified by :
walaa ghoniem

Patient Name : الأمثلة / ام الهنا عبدالمعطي بيومي عبدالمحسن	Request No. : 13007700
Patient No. : 27404010105141	Request Date : 16/04/2022
Path No. : 1018/2022	Verification Date : 26/04/2022
Sex : Female	Financial Category : اورام دار السلام - نفقة الدولة
Age : 48 Y	Department : Out Patient
Referring Doc. :	IHC No. :
	Registered by : .marwa mohamed yousse

IMMUNOHISTOCHEMISTRY REPORT

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PR: Strong nuclear reaction in 70% of tumor cells (score 8/8)
Her-2: Weak incomplete membrane reaction of tumor cells (score 1+)
Ki 67: 10 % nuclear reaction

Diagnosis:

Left breast, Tru cut biopsy;
- Invasive duct carcinoma, GII.
ER : Positive (8/8)

Verified by :

Dr.Marwa mahmoud

Printing Date : 08/05/2022 09:30:05

Page 1 of 2

Printed By : .marwa mohamed youss



خطابات أورام

NUCLEAR MEDICINE UNIT

وحدة الطب النووي

Patient Name: Om Elhana AbdElmoaty Bayomi.

Date: 17/8/2022

^{99m}Tc-MDP BONE SCAN

Desired information : skeletal surveying to detect multiple lesions.

Technique: Anterior and posterior whole body scan performed 2 hours after I.V administration of 20 mCi ^{99m}Tc-MDP:

Report:

- No definite hot spot that account for osseous deposits could be detected all over the surveyed skeleton.
- Degenerative changes are noted in the dorso-lumbar vertebrae.
- Both kidneys are seen

Conclusion:

- *No definite evidence of metastatic osseous deposits.*

Thanks for reference
Nuclear Medicine Unit

DR/Sherine Mahdy, MD